

Cannula vs Needle

When to use each, safety profile, technique comparison, and zone-by-zone recommendations

CANNULA VS NEEDLE – AT A GLANCE

CANNULA (BLUNT TIP)	NEEDLE (SHARP TIP)
Flexible, blunt-tipped — pushes vessels aside	Sharp — can penetrate vessels if not careful
Single entry point — multiple directions from one hole	Each deposit = new puncture site
Significantly reduced bruising and swelling	Higher bruising and swelling risk
Cannot penetrate vessel walls as easily	Can precisely target small anatomical structures
Requires larger entry needle (1 gauge up)	Direct access to target without prep needle
Slower technique — less precise placement for small deposits	Faster for multi-point small deposit techniques
Best for: large volume, high-risk vascular zones, linear threading	Best for: precise bolus, small deposits, structural supraperiosteal

ZONE-BY-ZONE RECOMMENDATION

Zone	Preferred	Rationale
Nasolabial fold	Cannula strongly preferred	Facial artery depth variability — cannula significantly reduces intravascular risk
Lips — body	Cannula preferred	Superior/inferior labial arteries — cannula approach from lateral entry
Lips — border definition	Needle (fine, 30g)	Precise intradermal threading along vermilion border requires needle accuracy
Cheeks — subcutaneous	Cannula preferred	Transverse facial and zygomatico-orbital vessels in this plane
Cheeks — supraperiosteal bolus	Needle (25–27g)	Direct bolus on bone — cannula less effective for deep structural placement
Tear trough	Cannula strongly preferred	Reduces Tyndall and vascular risk significantly. Infraorbital artery proximity
Temple	Cannula preferred	Superficial temporal artery — palpate pulse first, cannula medially
Jawline	Cannula preferred	Single entry, fan along mandible — efficient and lower bruising
Chin	Needle (25g)	Single supraperiosteal bolus — needle gives best precise periosteal contact
Nose (rhinoplasty)	Needle (27–30g)	Precise dorsal/tip placement — advanced injectors only
Forehead filler	Cannula strongly preferred	Supratrochlear artery risk — advanced technique only

CANNULA SIZING GUIDE			
Cannula Size	Filler Viscosity	Best Use	Entry Needle
18g	Very high G-prime (Voluma, Sculptra)	Deep structural cheek, jawline	18g or 16g needle
21g	High G-prime	Cheek, temple, jawline, body contouring	21g needle
22g	Medium G-prime	NLF, mid-face, general filler	22g needle
25g	Low-medium G-prime	Lips, perioral, tear trough	25g needle
27g	Very soft HA	Tear trough, fine lines, superficial	27g needle

ASPIRATION – DOES IT PREVENT VASCULAR OCCLUSION?

⚠️ Aspiration does NOT reliably prevent intravascular injection. Vessels are compressible — negative aspiration does NOT confirm extravascular placement.

✓ Aspiration is ONE tool in your safety toolkit — not a guarantee.

✓ Your true protection: anatomical knowledge + slow injection + small volumes + cannula in high-risk zones + hyaluronidase always in room.

★ If you see blood in the hub — stop. Apply pressure. Reassess before continuing.

TECHNIQUE TIPS – CANNULA	
Entry hole	Create entry with a sharp needle 1 gauge larger than cannula. Angle 45°, pop through dermis, then go horizontal
Direction	Always move cannula before injecting — inject while withdrawing (retrograde) not advancing
Feel for resistance	Resistance = tissue plane change. Reposition before injecting. Never force the cannula
No blood = safer	Blood in hub = intravascular — withdraw, apply pressure, reassess. Do not inject
Less is more	Deposit less volume per pass — you can always add. Overcorrection with cannula is harder to fix than with needle